

Mental Health, Stress, Workplace Productivity, And Worker Welfare

Absenteeism, Presenteeism, Stigma, Job Quality, And Causal Limits

Special Topic 6: Labor Market, Health, and Population – Week 4

Central question

- How do mental health and stress affect productivity, job quality, retention, and welfare inside labor markets?
- Mental health is not an individual residual; it is a labor-market force affecting attendance, effort, search, supervision, safety, and treatment.
- Week 4 discipline: distinguish mental health affecting work, work affecting mental health, treatment and stigma, and measurement and welfare.

Mental health as productivity and welfare object

Object	Labor margin	Welfare interpretation
Depression / anxiety	concentration, effort, social functioning	work may be more costly
Stress and pain	absences, errors, safety, fatigue	wage may miss burden
Substance use	attachment, reliability, search	dynamic scarring risk
Treatment	attendance, retention, recovery	access and timing matter
Job quality	autonomy, conflict, meaning	amenity and health object

- Wages and employment can miss absenteeism, presenteeism, stigma, safety risk, and diminished job quality.

Productivity margins inside work

- Absenteeism: missed days, sick leave, disability leave, and episodic nonattendance.
- Presenteeism: reduced concentration, speed, reliability, interpersonal functioning, or task completion while at work.
- Performance and safety: output, errors, customer interaction, supervisor ratings, and injury risk.
- Retention: quits, job-to-job moves, delayed promotion, occupational downgrading, and exit.
- Firm policy: scheduling, supervision, leave, accommodations, employee assistance, and monitoring.

Stigma, treatment, and disclosure

Mechanism	Worker response	Labor outcome
Stigma	hide symptoms or treatment	delayed support
Disclosure risk	avoid asking for help	no accommodation
Treatment friction	postpone care	absences or presenteeism
Manager beliefs	interpret distress as risk	promotion and retention effects
Career concerns	sort away from some jobs	constrained choice set

- Stigma is a labor-market mechanism because it changes information, help-seeking, accommodations, attendance, and careers.

Labor conditions shape mental health

- Workload and time pressure can raise stress and reduce recovery time.
- Autonomy and control can buffer stress by changing pace, methods, scheduling, and task order.
- Supervision and monitoring can provide support or generate anxiety and quits.
- Conflict, harassment, customer abuse, and discrimination can make work a direct source of mental-health harm.
- Isolation, remote work, mission alignment, and meaning affect support, identity, and welfare.

Measurement and causal challenges

Measure	What it captures	Main caution
PHQ-9, GHQ, CES-D, K6	symptoms and distress	differential reporting
Prescriptions / claims	treatment contact	access and stigma
Sick leave / disability	work disruption	program and workplace rules
Absenteeism	missed work	leave access and norms
Presenteeism	on-the-job loss	hard to observe cleanly
Performance / quits	workplace outcomes	job quality and selection

- Identification threats: reverse causality, dynamic selection, diagnosis intensity, treatment selection, co-morbidity, SES confounding, and reporting.

Frontier methods and research opportunities

- Treatment-access shocks: provider supply, coverage, copayments, employee assistance, and appointment access.
- Waiting-time shocks: delayed treatment as variation in recovery and labor attachment.
- Job-loss and workplace shocks: displacement, reorganization, monitoring, remote work, and pandemic-era changes.
- Policy changes: paid leave, flexibility, accommodation rules, disability rules, and mental-health coverage.
- Structural and job-search approaches: jointly model mental health, treatment, job choice, productivity, and welfare.

- Primary anchor: Bubonya, Cobb-Clark, and Wooden on mental health and productivity at work.
- Challenge anchor: job meaning, identity, and employee mental health.
- Reproduce: estimate a bounded synthetic profile linking distress to absenteeism, presenteeism, performance, quits, and welfare.
- Diagnose: separate productivity loss, job-quality effects, reporting, stigma, treatment selection, and reverse causality.
- Transfer: redesign the logic for treatment access, waiting time, job loss, workplace shocks, disclosure interventions, or structural search.

Research design checklist

- 1 Name the latent object: distress, diagnosis, treatment, functioning, stigma, or workplace loss.
- 2 Name the labor margin: attendance, presenteeism, performance, quits, promotion, safety, or welfare.
- 3 State the direction: mental health to work, work to mental health, or treatment and stigma to both.
- 4 Identify the main threat: reverse causality, selection, reporting, diagnosis intensity, co-morbidity, or SES.
- 5 Say what wages and employment miss.

- Week 4 centers mental health, stress, stigma, job quality, productivity, retention, and worker welfare inside workplaces.
- Week 5 scales up to disease exposure, environmental health, demographic change, and labor-market adjustment.
- The common thread is health as a force shaping workers, firms, allocation, productivity, and welfare.

Takeaways

- Mental health affects labor markets through productivity, attendance, search, retention, safety, treatment, disclosure, and welfare.
- Work conditions also produce mental-health outcomes through workload, autonomy, supervision, conflict, isolation, and meaning.
- Measurement is the core empirical problem: symptoms, treatment, claims, absences, and performance are different objects.
- The frontier is open because many estimates are correlational or structurally identified under strong assumptions.